

ORIGINAL ARTICLE

Health professions as strategic resources for community health: An innovative project proposal in an Italian Local Health Unit

TOMMASO PRIORESCHI¹, CRISTINA POGGI², CHIARA LORINI³

¹Department of Technical Health Professions, Local Health Unit Toscana Centro, Pescia, Italy; ²Department of Technical Health Professions, Local Health Unit Toscana Centro, Prato, Italy; ³Department of Health Sciences, University of Florence, Florence, Italy

ABSTRACT

Background: After COVID-19, Primary Health Care (PHC) has become essential for health system resilience. Despite investments from the National Recovery and Resilience Plan (PNRR), the use of epidemiological evidence in interprofessional PHC programmes remains limited. This study examines population health needs in the Toscana Centro Local Health Unit and proposes a governance model integrating epidemiological intelligence, interprofessional collaboration, and PHC.

Methods: The study used a two-phase design. Phase 1 consisted of a descriptive analysis of institutional epidemiological indicators (2019–2024) for contextual needs assessment. Phase 2 involved the co-design of a scalable Primary Health Care (PHC) model supported by the Council of Health Professions and the DRIADE digital platform, with targeted planning in the mountain municipality of San Marcello Piteglio.

Results: A substantial burden of non-communicable diseases linked to unhealthy lifestyles emerged, together with environmental pressures exceeding WHO thresholds—especially for NO₂ and O₃—and geographic disparities affecting rural and mountain areas. The proposed model integrates an interprofessional governance structure, continuous monitoring of health determinants, digital tracking of preventive and educational activities through DRIADE, and community-based interventions co-developed with local services and third-sector partners.

Discussion: The model supports the transition from primary medical care to PHC by linking epidemiological evidence with coordinated interprofessional strategies. It addresses challenges related to workforce variability, territorial fragmentation, and gaps in indicator availability.



Received: 21 November 2025 | Accepted: 7 May 2026

Correspondence: Tommaso Pioreschi, MSc TSM / Department of Technical Health Professions, Local Health Unit "Toscana Centro", Pescia, Italy / E-mail: tsrm-tommasopioreschi@outlook.com

Conclusions: An interprofessional PHC model grounded in epidemiological surveillance and digital integration may strengthen prevention, enhance health literacy, and improve citizen engagement, demonstrating feasibility in vulnerable territorial settings. (www.actabiomedica.it)

Key words: primary health care, public health workforce, community prevention, health governance, digital health, interprofessional collaboration

Background

Primary Health Care (PHC) has gained renewed strategic relevance in Italy, supported by the allocation of Next Generation EU (NextGenEU) funds to Local Health Districts (LHDs) and the establishment of the “Houses of Community” (HoC) (1). The COVID-19 pandemic highlighted the importance of PHC in safeguarding population health and strengthening community resilience. Health promotion and active citizenship, supported by education provided by health professionals, play a central role in raising awareness of health determinants and improving social empowerment (2).

The One Health approach emphasises the interconnection between human, animal, and environmental systems (3,4). Socio-economic disparities, environmental pressures, and waste management practices significantly influence health outcomes (5). Strengthening PHC contributes to greater equity and efficiency across health systems (6), with evidence showing reductions in health disparities where PHC structures are strong. In Italy, demographic ageing and the rising prevalence of chronic diseases continue to challenge the National Health Service (NHS) (7).

Continuous epidemiological surveillance and targeted interventions addressing behavioural risk factors are essential (8). International studies consistently show that robust PHC systems contribute to better population health outcomes (6,9).

This study assesses population health needs within the Toscana Centro Local Health Unit (LHU) and proposes an interprofessional PHC model informed by descriptive institutional data and territorial planning.

Methods

Phase 1: Data sources and analysis

Institutional and publicly accessible data were collected from Agenas, the Tuscany Region, the Ministry of Health, and the Toscana Centro LHU Department of Prevention (2019–2024) (10). Additional indicators were obtained from the Regional Health Agency of Tuscany (ARS Toscana) (11–13), the Tuscan Cancer Registry (Registro Tumori Toscano - RTT), and ISTAT. Variables included chronic disease prevalence, hospital utilisation, demographic trends, lifestyle factors, and environmental exposures (PM₁₀, PM_{2.5}, NO₂, O₃, heavy metals, and water quality). Data were sex-disaggregated whenever available and were validated by cross-checking multiple institutional sources. A focused analysis was conducted on San Marcello Piteglio, identified as a geographically and socially vulnerable municipality within the LHU (14).

Phase 2: From epidemiological needs assessment to project development

Italy's National Health Service (NHS) is organized in a regionalized Beveridge model (Law 42/2009) (15). Tuscany is divided into three LHUs—Toscana Centro, Nord-Ovest, and Sud-Est—and four University Hospitals. LHUs manage territorial care and ensure equity in service provision.

LHU Toscana Centro includes Florence, Prato, Empoli, and Pistoia. The study followed a two-phase design: (1) a descriptive cross-sectional analysis of institutional epidemiological and environmental indicators (2019–2024), conducted as a contextual needs

assessment; and (2) the co-design of a scalable intervention through the Health Professions Council (HPC), aligned with PNRR infrastructures and supported by the DRIADE system.

Special focus was given to San Marcello Piteglio, a mountain area characterized by vulnerability. Insights from general practitioners and local stakeholders informed strategic priorities. Data from ARS Toscana (2024) provided key indicators adjusted to minimize age bias across municipalities. A bubble diagram compared male and female populations; environmental indicators were analyzed with sex-disaggregated tables and graphs. No inferential statistical testing was performed, as the aim was not to examine associations between determinants and outcomes but to provide a contextual epidemiological needs assessment to inform the project proposal.

Based on these findings, a tailored project proposal was designed for the municipality with the poorest outcomes. Resources were linked to PNRR funding and existing allocations. Facilities identified for conversion included Territorial Operating Centers, Houses of Community, and Community Hospitals.

The Human Resources Department confirmed the presence of a broad range of the health professions recognized by Italian regulation. This professional plurality was incorporated into the project design as a strategic resource for community-based prevention, health promotion, education, and continuity of care (see Table 1).

In light of current literature (16), the strategic priority for LHUs is to disseminate a PHC culture aimed at fostering proactive, citizen-centred care. The strategy involves periodic analysis of local health determinants and reinforcement of primary, secondary, and tertiary prevention activities, as illustrated in Figure 2. These actions are grounded in an interdisciplinary approach integrating PHC, evidence-based medicine (EBM), evidence-based prevention, and evidence-based practice. To ensure project effectiveness, both clinical and economic governance are addressed. A solid foundation requires investment in human resources through continuous professional development and upskilling, promotion of care integration, and the delivery of safer, more appropriate interventions. Strengthening

digital systems will improve staff capabilities and support research and innovation.

Health promotion actions will be rooted in updated data streams from institutional databases. On the economic side, actions will aim to optimize processes, realign operations, and ensure budgetary sustainability.

Internally, Toscana Centro LHU will address system inefficiencies to improve service access and outcome effectiveness. Externally, the LHU will prioritize the needs of citizens and stakeholders, as stated in its corporate statute. Improving care quality will enhance the citizen-professional relationship, promote health literacy, and foster social empowerment and active citizenship within the PHC framework.

Results

Epidemiological needs assessment

Three priority needs emerged from the descriptive analysis of institutional epidemiological and environmental indicators: (1) high burden of NCDs with unfavorable lifestyle indicators; (2) environmental pressures exceeding WHO standards; and (3) geographic and social vulnerability in mountain municipalities. According to 2023 estimates by the National Institute of Statistics (ISTAT), life expectancy at birth in Toscana Centro LHU remains among the highest in Tuscany, peaking at 84.6 years in 2020 in Fiorentina Nord-Ovest area. As of January 1st, 2024, the Toscana Centro LHU counted 1,608,847 residents (48.5% male, 51.5% female), with Florence accounting for 22.6% of the total population. Despite a negative natural balance in 2023 (−8,848), the demographic trend is more stable than in other Tuscan LHUs.

Air quality data from ARPAT (2023) indicate that PM10 remained within legal limits, except in the Piana Lucchese station, flagged for criticality for the sixth consecutive year. NO₂ levels exceeded limits in the Florentine urban area, while O₃ targets were unmet in 40% of monitored stations. Though most values comply with national thresholds, they remain above WHO-recommended standards. Environmental issues are concentrated in the Florence-Prato-Pistoia plain.

Table 1. Listing of almost all the health professions present in the LHU, with references to their respective professional groups and their regulatory references.

Human Resource	Professional Group	Reference to the professional profile
Pharmacist	Health professions	D. LGS. 08.08.1991, N. 258
Medical doctor	Health professions	D. LGS. 17.08.1999, N. 368
Dentist	Health professions	L. 24.07.1985, N. 409
Veterinarian	Health professions	L. 08.11.1984, N. 750
Biologist	Health professions	L. 24.05.1967, N. 396
Physicist	Health professions	L. 11.01.2018, N. 3, ART. 8
Chemist	Health professions	R. D. 1.03.1928, N. 842 L. 11.01.2018, N. 3, ART. 8
Psychologist	Health professions	L. 18.02.1989, N. 56 L. 11.01.2018, N.3, ART. 9
Nurse	Nursing healthcare professions	D. M. 14.09.1994, N. 739
Pediatric nurse	Nursing healthcare professions	D. M. 17.01.1997, N. 70
Obstetrician	Midwifery health professions	D. M. 14.09.1994, N. 740
Radiographer	Technical health professions - diagnostic area	D. M. 14.09.1994, N. 746
Audiometric technician	Technical health professions - diagnostic area	D. M. 14.09.1994, N. 667
Biomedical laboratory technician	Technical health professions - diagnostic area	D. M. 14.09.1994, N. 745
Neurophysiopathology technician	Technical health professions - diagnostic area	D. M. 15.03.1995, N. 183
Prosthetist	Technical health professions - care area	D. M. 14.09.1994, N. 665
Acoustic-aid technician	Technical health professions - care area	D. M. 14.09.1994, N. 668
Cardiorespiratory physiology and vascular perfusion technician	Technical health professions - care area	D. M. 27.07.1998, N. 316
Dental hygienist	Technical health professions - care area	D. M. 15.03.1999, N. 137
Dietitian	Technical health professions - care area	D. M. 14.09.1994, N. 744
Podiatrist	Rehabilitation health professions	D. M. 14.09.1994, N. 666
Physiotherapist	Rehabilitation health professions	D. M. 14.09.1994, N. 741
Speech therapist	Rehabilitation health professions	D. M. 14.09.1994, N. 742
Orthoptist	Rehabilitation health professions	D. M. 14.09.1994, N. 743
Neurotechnologist and psychomotricity of the evolutionary age	Rehabilitation health professions	D. M. 17.01.1997, N. 56
Psychiatric rehabilitation	Rehabilitation health professions	D. M. 29.03.2001, N.182
Occupational therapist	Rehabilitation health professions	D. M. 17.01.1997, N. 136
Professional educator	Rehabilitation health professions	D. M. 08.10.1998, N.520
Prevention technician in the environment and workplace	Prevention health professions	D. M. 17.01.1997, N. 58
Health assistant	Prevention health professions	D. M. 17.01.1997, N. 69

Water quality data from 250 monitoring points show that in 2022, only 32% of rivers achieved “good” ecological status, with none rated “high”. The chemical status was “good” in 56% of locations, impacted by pollutants such as mercury and volatile organic

compounds. Surface water used for drinking showed criticalities: only one site (Florence) rated A1, while 51% fell into class A3 and 31% into SubA3 (10).

Lifestyle indicators show health challenges. Adult smokers in Toscana Centro LHU (24.7%) exceed the

regional average (22.9%), with 16.2% classified as heavy smokers. At-risk alcohol consumption affects 16.8% of adults (males: 20.2%; females: 13.5%). Teenagers show worrying trends: 52.3% report drinking alcoholic beverages, with higher prevalence among girls, while 24.3% report using illegal substances.

Diet and physical activity data highlight low adherence to health recommendations. In fact, only 11.5% of adults consume ≥ 5 daily portions of fruit/vegetables; among teenagers, the rate is 2.8%. Physical activity is lower than regional average (32.7% vs 34.1%), especially among females (27.2%). Sedentariness affects 30.5% of adults and is higher in girls (18.6%) than boys (8.2%).

Overweight affects 26.8% of adults (males: 34.0%; females: 19.8%) and obesity 7.5%, with consistent data across sexes and regions. Among adolescents, overweight affects 14.8% (males: 19.1%; females: 10.2%), and obesity is rare (males: 3.4%; females: 2.6%).

Mortality from diseases of the circulatory, respiratory, nervous, and neoplastic systems accounts for 71.7% of deaths in women and 74.9% in men.

According to the RTT, the most frequent (prevalence) cancers in Toscana Centro LHU are:

- women: breast (29%), colorectal (12%), lung (8%);
- men: prostate (19%), lung (15%), colorectal (13%).

Among younger men (<50 years), melanoma (16%), testicular (10%), and thyroid cancers (9%) are most frequent; among women: breast (39%), thyroid (15%), and melanoma (11%). In older women (50+), colon and lung cancers become more prevalent.

Sex-disaggregated indicators for San Marcello Piteglio showed different patterns according to the outcome considered. Higher values were observed among males for COPD, mortality due to circulatory diseases, and chronic dementia, whereas Table 3 reported higher values among females for all-cause mortality, deaths from cancer, and mortality from respiratory diseases. The ageing index was similar across sexes, although life expectancy remained higher among women. Given the descriptive nature of the study and the heterogeneity of indicator definitions, these data were used for contextual prioritization and not for causal interpretation.

Age-standardized cancer mortality rates by sex and tumor site showed relevant territorial differences across Tuscany. In the Pistoia district, mortality rates for lung, colorectal, and stomach cancers were higher among males than females, as reported in Table 2. Sex-disaggregated health indicators for San Marcello Piteglio are reported in Table 3.

In this context of complex and evolving epidemiological patterns—especially non-communicable diseases, chronic conditions, and demographic ageing – an interprofessional approach is particularly relevant. In this perspective, the project proposal includes a broad heterogeneity of health professions, which are already part of the healthcare staff of the Toscana Centro LHU. This represents a strategic asset, as it enables the system to address the full spectrum of population health needs across the preventive continuum, from primary to secondary and tertiary prevention. Such plurality of competencies allows the construction of integrated pathways in which each profession contributes according to its institutional mandate and specific scope of practice, without overlapping responsibilities. Rather than concentrating preventive responsibility within a single professional domain, the project activates complementary expertise already embedded within the organisation to build a governance-oriented and integrated PHC model.

Even professionals who do not formally hold a specific institutional mandate for primary prevention—such as the Neurophysiopathology Technician or the Radiographer—can meaningfully support the project through activities related to secondary and tertiary prevention, including screening programmes, early diagnosis, chronic disease monitoring, follow-up pathways, and patient education initiatives. In this perspective, prevention is conceptualized as a continuum of care rather than the exclusive prerogative of a single professional profile.

Project proposal

Findings from the epidemiological needs assessment indicated that the population living in the mountain municipalities located within the Toscana Centro LHU, and particularly in the municipality of San Marcello Piteglio, had worse health indicators.

Table 2. Age-standardized cancer mortality rates by sex and tumor site in Tuscany Region (Regione Toscana, RT), 2018–2020.

Male										
Tumor site	Area									
	Pistoiese	Valdinievole	Pratese	Fiorentina	Fiorentina Nord	Fiorentina Sudest	Mugello	Empolese Val d'Arno	Toscana Centro LHU	RT
Lung	76.1	83.9	78.8	64.5	73.1	65.4	67.9	74.9	72.2	71.7
Colonrectal	39.3	34.8	40.3	31.0	32.5	32.2	28.8	35.6	34.6	34.8
Stomach	17.7	23.7	21.0	17.9	21.1	18.7	30.4	15.9	19.5	19.0
Prostate	20.2	24.0	23.4	25.5	23.3	28.2	25.1	26.3	24.6	25.1
Female										
Tumor site	Area									
	Pistoiese	Valdinievole	Pratese	Fiorentina	Fiorentina Nord	Fiorentina Sudest	Mugello	Empolese Val d'Arno	Toscana Centro LHU	RT
Lung	30.4	25.3	28.2	29.9	24.0	29.0	28.6	23.8	27.6	27.0
Colonrectal	20.8	21.8	18.7	20.4	22.3	19.6	24.4	23.5	21.0	21.2
Stomach	7.5	6.8	9.7	7.3	10.4	8.8	17.1	6.5	8.5	8.5

Table 3. Health status indicators in San Marcello Piteglio, by sex.

Diseases	Percentage distribution of diseases among males	Percentage distribution of diseases among females
Hospital discharge for all causes	86.44	24.17
Chronic copd patients	67.03	34.79
Mortality due to circulatory diseases	64.83	34.79
Chronic dementia patients	71.42	35.89
Chronic diabetes mellitus patients	46.15	38.82
Chronic heart failure patients	39.92	42.22
All-cause mortality	53.11	63
Deaths from cancer	64.46	73.62
Mortality from respiratory diseases	32.23	86.44
Life expectancy at birth	61	90
Ageing index	93.4	93.04

For this reason, we decided to tailor the project proposal to that area.

The resources required to implement the project proposal will be provided by the financial funds allocated to the PNRR. Any additional economic resources can be found using what is already in existence in Toscana Centro LHU. These include the possible expenditure to hire other personnel related to the Department of Health Technical Professions, as well as the provision of technological resources to carry out home care. This has to be considered as an investment, with a return in terms of health and well-being of the population concerned, consistent with the LHU's mission (17), contained in the LHU's statutes.

According to previous decisions by the top management of the Toscana Centro LHU, the Community Hospital and the House of Community will be located in the study area (18).

For the project to be effective, it will be necessary to implement a good synergy between economic, technological, structural and, finally, professional resources. It is useful to maximise efficiency in each part of the project to ensure that the main objective is achieved: the transition from a health care culture based on primary medical care to one based on PHC.

To achieve these goals, an interprofessional group should be established, including medical and non-medical health professions, named the Health Professions

Council (HPC). The HPC would aim to integrate prevention and health promotion pathways for citizens, improve population health, and disseminate a PHC culture throughout the LHU. The HPC would be responsible for devising, designing, implementing and monitoring over time the development of integrated prevention and promotion of citizens' health. The educational needs of the population would be identified by highlighting specific health needs emerged from the periodical analysis of health data, which is drawn up annually by the Department of Prevention of the LHU.

The HPC would establish organisational and hierarchical links with various organisational structures and public bodies, including professional orders, Houses of Community, third-sector organisations, and corporate departments. It would be hierarchically subordinate to the Board of Health (Consiglio dei Sanitari) (17), to which it would report on its activities in terms of efficiency and effectiveness.

One of the objectives of the HPC would be to bring professionals closer to citizens, so it is vital that its activity would take place within the community, collaborating with both the Houses of Communities and the third sector. The latter could act as a link between the various public and private bodies present in the area. It also includes all non-governmental associations, which represent an emotional and psychological bridge between citizens and health institutions.

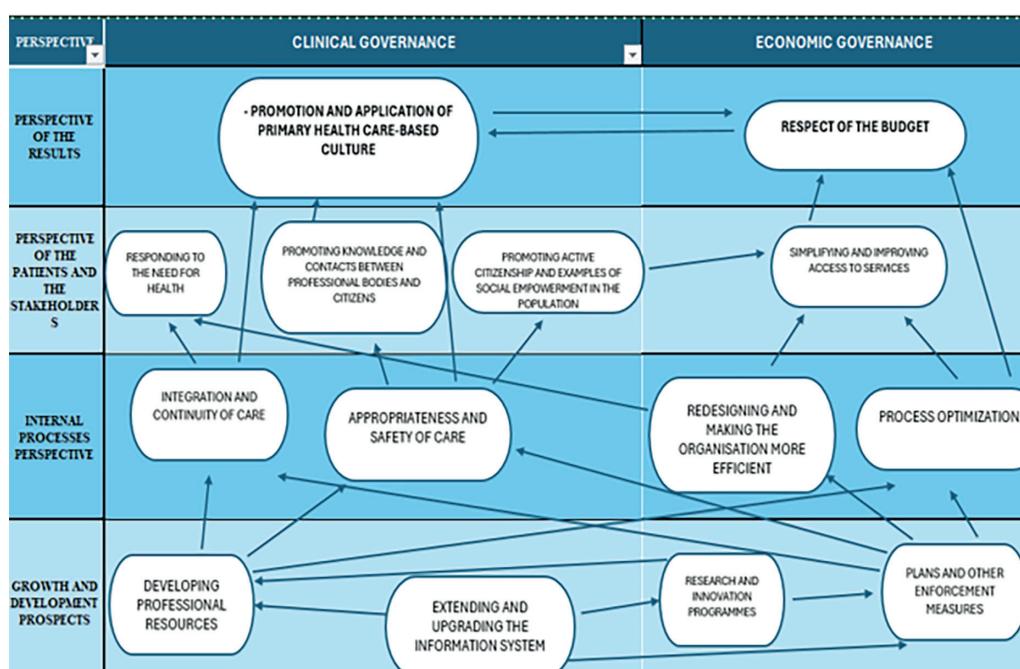


Figure 1. Strategy map for project implementation. The perspectives and their typologies are shown in the left column, through which clinical and economic governance are articulated.

To this end, the health professions would collaborate in organizing events open to the public with the aim of introducing the health professions to the audience, raising awareness and bridging the gap between the population and the health system. Any health promotion intervention would take place in the community and with the community. For this reason, the participation of each health profession would be fundamental to involve the different professional profiles.

The HPC would have a direct line with each corporate department to which the various professions belong; this would ensure consistency and appropriateness with the objectives assigned to the different organisational structures. The partnerships described are expected to strengthen appropriateness and collaboration between public bodies and third-sector organisations, while supporting the project implementation strategy in terms of both clinical and economic governance (Figure 1).

Collaboration and partnership between different public and private bodies would help to achieve the expected results of social empowerment and active citizenship phenomena in the community itself.

Each health profession would respond to population health needs according to its competencies across primary, secondary, and tertiary prevention, as well as diagnosis and care, from either single-profession or interprofessional perspectives. In any case, empowerment and health literacy should be among the main goals of the interventions, encouraging individuals to shift from service users to active agents of community change (19), as illustrated in Figure 2.

A new digital platform, named DRIADE, should be developed to allow different professionals to interact and share information about citizens involved in healthcare, disease prevention, and health promotion interventions. The characteristics of this platform should make it possible to manage the flow of data relating to the activities of the individual patient at home, the classic service activities and, finally, health education and promotion activities. These latter activities will be like those normally carried out by professionals but would aim to promote a culture of prevention and health education among the population. The processes associated with these activities would also be implemented at patient's home, to make the home care approach as concrete as possible, as indicated in the

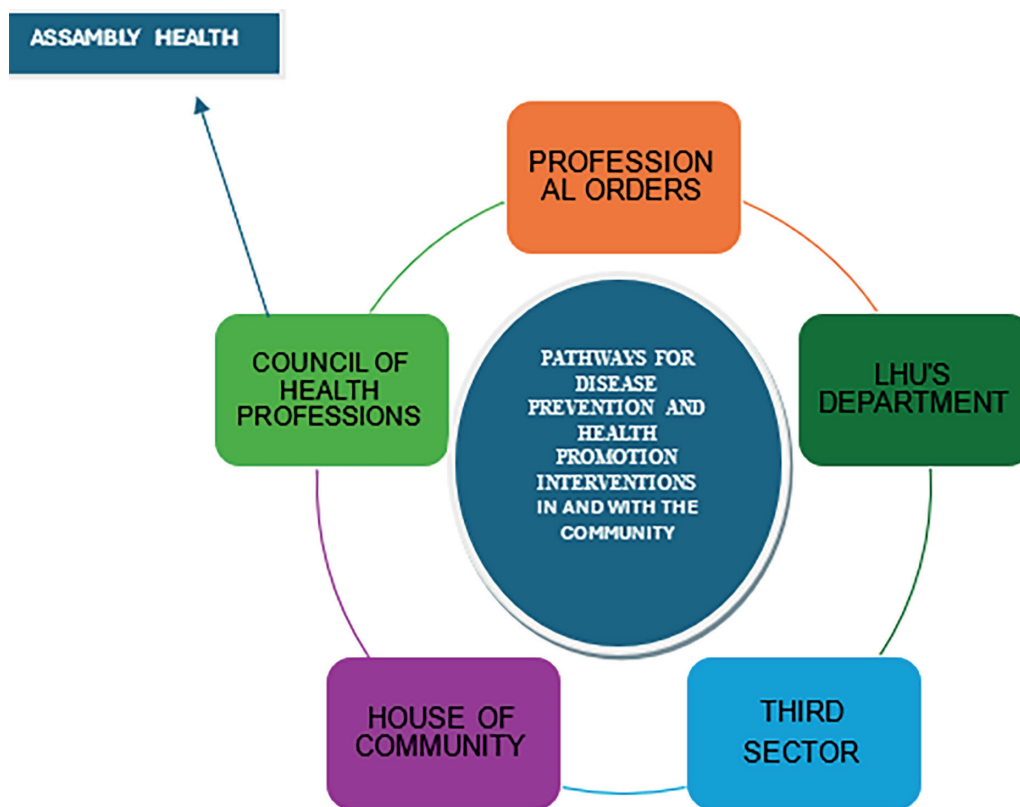


Figure 2. Collaboration between public bodies and third sector for health promotion.

Resilience and Recovery Plan. According to this approach, the patient's home would not only be a place of care, but also a place of health promotion.

The possibility of interaction between professionals could be a valuable resource for care and would allow a personalized and integrated approach. In addition, tracing the health education and promotion activity to which a citizen has participated would allow information to be collected.

Citizens should register in DRIADE the health promotion initiatives they will attend; interactions with different health professionals would be provided, so to get further information on particular topics. DRIADE could feature a dedicated space for in-depth insights into the roles of different healthcare professionals.

Data registered in DRIADE would be periodically analyzed and discussed by the Health Professions Council (HPC), as well as by professionals working in the Houses of Community. Each of these actors would manage a small part of the data flow.

Discussion

This paper aims at describing a project proposal to enhance PHC response to health need in a certain area in Tuscany. The proposed model emphasizes interprofessional responsibility in integrating primary and hospital care. By explicitly linking determinants to interprofessional actions, the model operationalizes the shift from Primary Medical Care to PHC. Compared with several European peers, where prevention investment and advanced nursing roles are more established, the Italian PNRR provides a unique opportunity to re-balance resources toward community-based prevention.

Following the COVID-19 experience, such an approach is crucial to ensure continuity in chronic care and territorial services (20). Each health profession must embrace its role also as educator and promoter of healthy lifestyles, particularly in light of the increasing prevalence of non-communicable diseases and demographic shifts.

Barnett et al. (21) depicted the typical chronic patient as elderly, affected by multiple pathologies and often comorbid with psychological disorders, linking multimorbidity to low PHC system performance. Non-communicable diseases have become a growing social issue (22,23), and trends such as aging and rising life expectancy suggest that their burden will intensify. Investing in PHC is therefore a strategic necessity. As shown in Costa Rica, PHC-oriented systems can reduce NCD mortality among older adults (24). Recent studies underline that addressing social needs in PHC requires balancing two different perspectives. In fact, *value-based care* focuses on cost-effectiveness and measurable outcomes, assessing whether an intervention is efficient and financially sustainable. In contrast, *values-based care* reflects the ethical and professional duty to act even when interventions improve health but are not cost-saving. Basu et al. illustrate the financial limits of current social-needs programmes, while Berkowitz et al. highlight that many of these interventions remain essential because they embody the core values of the health professions and public health practice (25,26).

Health professionals contribute to disease prevention and health promotion not only in acute-care settings, but also in community contexts. In Toscana Centro LHU—particularly in rural municipalities like San Marcello Piteglio—chronic patients are predominantly elderly and multimorbid. The project aligns with the LHU's 2022–2024 performance plan, which is built around territorial health indicators (27).

In Italy, NHS funds are allocated 51% to territorial care, 5% to prevention, and 44% to hospitals. By contrast, Germany allocates only 1.6% to prevention, while Sweden, France, and Spain have stronger PHC structures, with integrated services and expanded nursing roles (28,29). According to WHO, 80–90% of NCD deaths occur in low- and middle-income countries, with 29% before age 60 (30).

The inclusion of international experiences and comparative European data was not intended to suggest direct transposition of foreign organizational models into the Italian context. Rather, the international background was analyzed to situate the project within the broader scientific and policy debate on PHC, and to identify structural gaps and strategic opportunities within the national system.

Comparative evidence from countries with stronger PHC-oriented systems provides a conceptual framework to interpret the epidemiological challenges observed in the Toscana Centro LHU and to justify the need for interprofessional and governance-based innovation. In this sense, the international perspective serves as a benchmarking and interpretative tool, while the proposed intervention remains context-specific and aligned with the Italian regulatory and organizational structure.

The Toscana Centro LHU employs all 30 health professions recognized in Italy, contributing not only to care but also to education and health promotion (31). Their interprofessional collaboration is key to driving the cultural transition toward Primary Health Care.

The study's limitations include partial inconsistencies in indicator availability across the time span due to differences in data collection and processing methods. Nevertheless, the observed trends appear coherent and aligned with regional patterns. Challenges to implementation may stem from delays in accessing National Recovery and Resilience Plan (NRRP) funds and workforce fluctuations, including retirements, turnover, and unfilled positions.

Conclusions

The project proposal strategy aligns with the financial balance required for the delivery of Essential Levels of Care (Livelli Essenziali di Assistenza – LEA), aiming to optimize resource use, improve outcomes, and support behavioral change. These goals would be pursued through continuous management of chronic diseases, structured prevention, and health education. Such measures should enhance participation in screening campaigns and encourage healthier lifestyles, thus strengthening public health strategies.

Toscana Centro LHU hosts all 30 health professions recognized in Italy. The project proposal suggests the development and implementation of new structural tools, such as the HPC—an institutional body dedicated to supporting territorial health promotion starting from a population needs analysis—and the DRIADE information system, devoted to both professionals and citizens. These instruments aim to reinforce

health literacy, interprofessional collaboration, and social empowerment, promoting a sense of shared responsibility and active community engagement.

The epidemiological profile of the region highlights the urgency of addressing the prevalence and mortality of non-communicable diseases. Social empowerment mechanisms, supported by robust health literacy processes, can serve as transformative tools in this context. Among the most relevant expected outcomes is the emergence of active and aware citizenship.

Health professions, already proven to be a strategic resource—particularly during the COVID-19 pandemic—will likely play a leading role in the cultural shift from Primary Medical Care to PHC. Moving beyond professional silos toward interprofessional collaboration will help establish integrated care pathways where prevention, health promotion, and initiative medicine reshape the citizen–health system relationship.

This will lead not only to a valorization of the health professions, but also to a different vision of them: not merely as operators of a service for the community, but an important part of it. This proposal suggests that health professions, through interprofessional governance and digital integration, can help reorient the NHS from service-centred delivery to community-centred PHC, turning prevention and empowerment into routine practice.

Ethical Considerations: The data analyzed in this study were freely accessible and available on institutional websites or in the online epidemiological databases mentioned above. No Ethics Committee approval was required for this study, as all data were publicly available and accessible through institutional sources.

Conflict of Interest: The authors declare no conflict of interest. The authors also declare that the project proposal described in this paper does not necessarily reflect the official views of the Toscana Centro Local Health Unit or the Tuscany Region.

Authors' Contribution: TP conceived the study, collected and curated the data, performed the descriptive analysis, developed the project proposal, and drafted the manuscript. CP contributed to the project design, professional and organizational framing, and critical revision of the manuscript. CL supervised the methodological framework, contributed to the public health interpretation, and critically revised the manuscript. All authors read and approved the final version of the manuscript.

Declaration on the Use of AI: Artificial intelligence (AI)–assisted technologies (ChatGPT, OpenAI, 2025 version) were used exclusively for language refinement and bibliographic formatting. The use of AI was strictly limited to these aspects, while all scientific contents, results, and interpretations are the sole responsibility of the authors, who verified and approved every section of the manuscript.

Funding: This research received no external funding.

References

1. Maciocco G, Brambilla A. Prefazione. In: Dalle Case della Salute alle Case della Comunità. La sfida del PNRR per la sanità territoriale. Roma: Carocci Editore; 2022:11-3.
2. Milani C, Naldini G, Baggiani L, Nerattini M, Bonaccorsi G. How to promote changes in primary care? The Florentine experience of the House of Community. *Front Public Health*. 2023 Sep 5;11:1216814. doi: 10.3389/fpubh.2023.1216814
3. Zinsstag J, Crump L, Schelling E, et al. Climate change and One Health. *FEMS Microbiol Lett*. 2018;365(11):fny085. doi: 10.1093/femsle/fny085
4. De Macedo Couto R, Brandespin DF. A review of the One Health concept and its application as a tool for policy-makers. *Int J One Health*. 2020;6(1):83-9. doi: 10.14202/IJOH.2020.83-89
5. Calman NS, Golub M, Shuman S. Primary care and health reform. *Mt Sinai J Med*. 2012;79(5):527-34. doi: 10.1002/msj.21339
6. Starfield B, Shi L, Macinko J. The primary care imperative: reshaping health systems for greater equity. *Milbank Q*. 2005;83(3):457-502. doi: 10.1111/j.1468-0009.2005.00409.x
7. Istituto Nazionale di Statistica. Natalità e fecondità della popolazione residente – anno 2022. Roma: ISTAT; 2023. Available from: <https://www.istat.it/it/archivio/289772> [Accessed 2023 Oct 26].
8. Alti E, Mereu A. Community and Primary Care. In: Textbook of Patient Safety and Clinical Risk Management. Cham: Springer; 2021:3-5.
9. Maciocco G. Cure primarie e servizi territoriali: esperienze nazionali e internazionali. Roma: Carocci Editore; 2021:22.
10. Azienda USL Toscana Centro. Stato della salute e della sicurezza umana. Available from: <https://www.uslcentro.toscana.it/index.php/amministrazione-trasparente-2/informazioni-ambientali/21011-stato-della-salute-e-della-sicurezza-umana> [Accessed 2024 Sep 14].
11. Agenzia Regionale di Sanità della Toscana. Attività e pubblicazioni. Available from: <https://www.ars.toscana.it/> [Accessed 2023 Jan 4].
12. Agenzia Regionale di Sanità della Toscana. Indicatori socio-sanitari in Toscana. Available from: <https://www.ars.toscana.it/indicatori-socio-sanitari> [Accessed 2024 Sep].

13. Agenzia Regionale di Sanità della Toscana. PrOTer-MaCro: Informazioni su processi, esiti di salute e costi sostenuti nella cura delle persone con cronicità. Available from: <https://www.ars.toscana.it/proter-macro> [Accessed 2024 Sep].
14. Agenzia Regionale di Sanità della Toscana. La salute dei comuni: San Marcello Piteglio. Available from: <https://www.ars.toscana.it/banche-dati/dati-sintesi-sintcomuni-san-marcellopiteglio> [Accessed 2024 Sep 14].
15. Italia. Legge 5 maggio 2009, n. 42. Delega al Governo in materia di federalismo fiscale. Gazzetta Ufficiale della Repubblica Italiana. 2009 May 5;105.
16. Sawin G, O'Connor N. Primary care transformation. *Prim Care*. 2019;46(4):549-60. doi: 10.1016/j.pop.2019.07.003
17. Azienda USL Toscana Centro. Statuto aziendale. Available from: <https://www.uslcentro.toscana.it/index.php/amministrazione-trasparente-2/assetto-organizzativo/32979-statuto-aziendale> [Accessed 2026 Mar 8].
18. Ministero della Salute. I dipartimenti ospedalieri nel Servizio Sanitario Nazionale. Roma: Ministero della Salute; 2007.
19. Zanobini P, Del Riccio M, Lorini C, Bonaccorsi G. Empowering sustainable healthcare: the role of health literacy. *Sustainability*. 2024;16(10):3964. doi: 10.3390/su16103964
20. Profili F, Maciocco G, Bellini B, et al. Prevalence and clustering of chronic diseases in Tuscany (Central Italy): evidence from a large administrative data warehouse. *Epidemiol Prev*. 2020;44(5-6):385-393. doi: 10.19191/EP20.5-6.P385.090
21. Barnett K, Mercer SW, Norbury M, et al. Epidemiology of multimorbidity and implications for health care, research, and medical education. *Lancet*. 2012;380(9836):37-43. doi: 10.1016/S0140-6736(12)60240-2
22. Genovese C, De Belvis AG, Rinaldi M, et al. Quality and management care improvement of patients with chronic kidney disease. *J Prev Med Hyg*. 2018;59(4):E305-E310. doi: 10.15167/2421-4248/jpmh2018.59.4.999
23. Lenzi J, Avaldi VM, Rucci P, Pieri G, Fantini MP. Burden of multimorbidity in relation to age, gender and immigrant status. *BMJ Open*. 2016;6(12):e012812. doi: 10.1136/bmjopen-2016-012812
24. Mora-García CA, Pesec M, Prado AM. *The effect of primary healthcare on mortality: Evidence from Costa Rica*. *J Health Econ*. 2024;93:102833. doi: 10.1016/j.jhealeco.2023.102833.
25. Basu S, Berkowitz SA, Davis C, Drake C, Phillips RL, Landon BE. Estimated Costs of Intervening in Health-Related Social Needs Detected in Primary Care. *JAMA Intern Med*. 2023 Aug 1;183(8):762-74. doi: 10.1001/jamainternmed.2023.1964
26. Berkowitz SA, Baggett TP, Edwards ST. Addressing health-related social needs: value-based care or values-based care? *J Gen Intern Med*. 2019;34(9):1916-8. doi: 10.1007/s11606-019-05087-3
27. Azienda USL Toscana Centro. Piano della Performance Aziendale 2022-2024. Firenze: USL Toscana Centro; 2022. Available from: <https://www.uslcentro.toscana.it/index.php/performance/32979-piano-della-performance-2022-2024> [Accessed 2024 Oct 11].
28. Agenas. Analisi comparata delle cure primarie in Europa. In: *Monitor*. 2022;2(Suppl):1-15.
29. Anell A, Glenngård AH, Merkur S. Sweden: health system review. *Health Syst Transit*. 2012;14(5):1-159. PMID: 22894859
30. World Health Organization. Noncommunicable diseases: Executive summary. Geneva: WHO; 2021.
31. Federazione Nazionale Ordini TSRM e PSTRP. Costituzione etica. Roma: FNO TSRM e PSTRP; 2021.

Copyright: The Author(s), 2026. Licensee Mattioli 1885, Fidenza, Italy. This is an open-access article distributed under the terms of the Creative Commons Attribution NonCommercial License (CC BY-NC-4.0).

Disclaimer/Publisher's Note: The statements, opinions and data contained in this article are solely those of the author(s) and contributor(s) and do not necessarily reflect those of their affiliated organizations, the publisher, the editors or the reviewers. The publisher and the editors disclaim any responsibility for injury to people or property resulting from any ideas, methods, instructions or products mentioned in the content. Any product that may be evaluated in this article, or claim made by its manufacturer, is not guaranteed or endorsed by the publisher.